

DS2444B: The Medical Report, a completion guide

A clinical, question-by-question guide for the health professional completing the disability medical report, with suggested wording keyed to the eligibility test. September 2026.

Before you use any wording on these pages

Every block here is scaffolding, to be completed to the true clinical picture and no further. Delete any clause that does not fit this patient.

Accuracy is the strongest position and the one that withstands review. An overstated report can be challenged, and it can cost a patient the case they truly have.

Your signature certifies your clinical opinion. These blocks exist to put your real findings into the language the determination weighs, not to supply findings you did not make.

For the completing health professional

This is a shorthand guide to the DIA Medical Report (DS2444B). It maps each field to what the adjudication turns on, and offers suggested wording keyed to the eligibility test. The official form is at open.alberta.ca.

The test the report must speak to

AISH eligibility: a severe and permanent impairment that substantially limits the ability to earn a livelihood. Document the limitation and its permanence, not the accommodations that would let the patient work.

The accommodation trap: the single most important framing

Accommodation language reads as capable with support, which is the ADAP finding rather than the AISH one. Where clinically true, state that no accommodation renders competitive employment reliable or sustainable.

Unpredictability and inability to self-sustain are often the strongest findings. An employer can adapt a workstation; none can adapt to unpredictable incapacity.

Relationship with applicant (Q1 to Q4)

Q1: confirm Alberta registration; give discipline and registration number. Q2 to Q4: treating duration, last contact, and frequency. Longer, regular treating relationships carry evidentiary weight, so state them plainly (for example, primary provider since [yyyy], reviewed [monthly]). A specialist seeing the patient episodically should say so; it frames the scope of the opinion.

Diagnosis and medical history (Q5 to Q10)

Q5. Diagnoses, codes, onset. List all conditions, primary and secondary, with codes and onset dates. Early or congenital onset dates support permanence, so record them precisely. Include comorbidities that compound function even if individually sub-severe; the cumulative picture is what is adjudicated.

Q6. Diagnosis detail and specific symptoms. Etiology, classification, stage or grade, and the specific symptoms per diagnosis.

Suggested wording, Q6

[Condition]: [etiology / classification]. Symptoms this patient experiences: [list]. Functional consequence: [what the symptoms prevent, for example sustained concentration, reliable attendance, self-initiation of tasks]. Onset [mm-yyyy]; course [static / progressive / relapsing].

Q7. Medical history and chronology. This is the central narrative field. Tie each condition to a duration and a functional restriction, and close on permanence and impact on earning capacity.

Suggested wording, Q7

[Patient] has a [X]-year documented history of [condition(s)], first presenting [mm-yyyy], with a [stable / progressive] course despite [treatments tried]. The impairment restricts [functions]. These restrictions are [continuous / unpredictable and recurrent] and are not resolved by workplace accommodation, because [reason, for example the limiting factor is unpredictable incapacity rather than the work environment]. In my opinion the condition substantially limits this patient's ability to earn a livelihood on a reliable, sustained basis.

Q8 to Q10. Referrals, evidence, admissions. List specialist consultations and attach reports; attach diagnostics, imaging and labs; list relevant hospital admissions with dates and reasons. Attaching documents pre-empts a later request for further information.

Degree of impairment: activities of daily living (Q11 to Q40)

Rate each item to the true clinical picture: 0 none, 1 mild, 2 moderate, 3 severe or unable. Do not under-rate to appear conservative. An accurate 2 or 3, with a one-line note in the describe column, carries the file.

Note variability, and do not average it away

Adjudication weighs cumulative burden across domains. Complete the describe box wherever a rating is 2 or 3.

For episodic and fluctuating conditions, write the range rather than the mean: 3 on flare days, 1 between, not a blended 2.

The government's own guide acknowledges that recurring conditions can cumulatively amount to severe disability. A good day is not the measure; the pattern is. Averaging erases exactly the evidence that matters.

Suggested wording, Q40

Across mobility, cognition, personal care and independent living, this patient's cumulative impairment is [moderate to severe]. Key drivers: [items rated 2 to 3]. On [flare / bad] days, [patient] requires [assistance / supervision] for [tasks]. The combined effect is that [patient] cannot independently maintain the routine and reliability that sustained employment requires.

Level of impairment: mental health and IEWS (Q41 to Q68)

The same rating discipline, 0 to 3, across mood, thought and perception, cognition, behaviour, orientation, speech, somatic and safety. The fields that most often carry these files: Q50 to Q52 attention, concentration and comprehension; Q53 executive function, which is pivotal for acquired brain injury and neurodevelopmental profiles; Q57 memory, specifying long, short or working; Q60 amotivation, Q59 impulse control and Q43 emotional dysregulation, describing the functional cost in each case. Q67, suicidality, is a clinical safety field; complete it from your assessment as you would any risk documentation.

Suggested wording, Q68

Cumulative mental-health impairment is [moderate to severe], driven by [items rated 2 to 3]. [Executive dysfunction / cognitive deficits / mood instability] mean [patient] cannot reliably [initiate and sequence tasks / sustain attention / regulate under ordinary workplace demand] without continuous external structure. These deficits are [longstanding / organic] and persist [despite treatment]. They are not correctable by accommodation, as they affect the capacity to self-organise and self-sustain that any employment presupposes.

Remedial therapy (Q69 to Q74): the permanence gate

Denials often turn on the theory that more therapy might improve the condition, so it is not yet permanent. Answer these fields so the record closes that gate where it is clinically closed: treatments tried and exhausted, and why further treatment is not expected to restore work capacity. If a treatment is nominally available but inaccessible in a clinically relevant timeframe, such as a waitlist of 12 to 16 months or more, say so. It is not meaningfully available.

Q69 to Q70. Medications and their functional effect, including sedation or cognitive side effects that themselves limit work; treatment history with timeframes and outcomes.

Q71. Current plan, when initiated, review frequency, or, if options are exhausted, say so explicitly.

Suggested wording, Q71

Current management: [plan], initiated [mm-yyyy], reviewed [interval]. Available interventions have been [tried / exhausted] with [partial / no] functional gain. [Remaining options are limited to symptom management, not restoration of work capacity. / Further options are inaccessible within a clinically relevant timeframe: [waitlist / access barrier].]

Q72 to Q73. Compliance. Answer truthfully. An unexplained non-compliant can sink an otherwise strong file. Legitimate reasons belong on the record.

Suggested wording, Q72 to Q73

Adherence is limited by [cost / side-effect intolerance / access / the disability itself, for example executive dysfunction impairing medication management]. This is a consequence of the condition and circumstances, not refusal of care.

Q74. Further treatment anticipated? If it is not expected to restore the capacity for sustained work, state that plainly.

Suggested wording, Q74

No further treatment is anticipated that would be expected to restore this patient's capacity for reliable, sustained employment. Management is directed at [symptom control / stability], not functional recovery.

Prognosis (Q75): the determinative field

Four options: Temporary, Episodic, Permanent, Undetermined. Where clinically supported, Permanent, not expected to change or improve over time with treatment, is the finding the AISH determination rests on. For episodic conditions, use Episodic and quantify frequency, length, severity and total duration; unpredictable recurrence is itself a barrier to employment. Avoid Undetermined if the record supports permanence, since an ambiguous prognosis defaults the file toward the lower program.

Suggested wording, Q75

Permanent. [Condition(s)] are longstanding and not expected to improve with further treatment. Prognosis for return to reliable, sustained employment is poor. [For episodic: episodes occur [frequency], last [duration], severity [level], onset unpredictable; between episodes the patient [residual limitation].]

Additional comments (Q76): the closing synthesis

Draw the domains together into one statement of cumulative impairment, permanence, and impact on the ability to earn a livelihood.

Suggested wording, Q76

In summary, [patient] lives with [severe and permanent condition(s)] producing cumulative impairment across [physical / cognitive / mental-health] domains. The limitations are [continuous / unpredictable], not correctable by workplace accommodation, and not expected to improve with treatment. In my clinical opinion, [patient] is substantially limited in the ability to earn a livelihood on any reliable, sustained basis.

Records to draw on, and certification

Netcare is not the full record. Alberta Netcare holds labs, imaging, dispensed medications and hospital records, not community clinic office notes. Where relevant history sits outside Netcare, draw on the clinic chart, pharmacy record and specialist correspondence so the report is complete rather than partial.

Certification. Completing the report certifies that it contains your clinical findings and medical opinion. Provide name, CPSA or CRNA number, contact details, date and signature.

The Alberta Disability System Breakdown is a free community campaign, not a government body, and receives no government funding.
albertadisabilitybreakdown@outlook.com albertadisabilitysystembreakdown.netlify.app General information, not legal or medical advice. Free to share and print.